

Adrabetadex Coding, Reimbursement, and Provider Economics Training Guide

Audience: Area Reimbursement Managers

Purpose: Prepare ARMs to explain Adrabetadex reimbursement mechanics, launch-period coding expectations, OPPS pass-through timing, 340B economics, sourcing pathways, Medicaid considerations, and provider net-cost recovery.

Executive Summary

Adrabetadex launch adoption is expected to be **COE-led**, with **340B-eligible hospital outpatient departments** as the default planning scenario. Non-340B pathways should remain in the model, but as sensitivity scenarios rather than the primary first-wave launch story.

Coverage alone does not make administration financially sustainable. A treatment center still needs to understand whether it can acquire the product, obtain authorization, code and document the claim correctly, receive timely payment, and manage operating burden without recurring loss or unacceptable cash-flow exposure.

Core ARM Message

Adrabetadex is not only a coverage conversation. It is a provider economics, coding-readiness, and operational-readiness conversation.

Planning Assumptions

Item	Planning Assumption	ARM Relevance
WAC per 900 mg vial	\$39,000	Establishes acquisition exposure
Dosing frequency	Every 2 weeks	Creates 26 annual administrations
Annual administrations	26	Defines recurring annual treatment burden
Annual gross WAC exposure	\$1,014,000	Frames patient-level working-capital exposure
First two quarters exposure	\$507,000	Frames launch-period pre-ASP exposure
Initial Medicare benchmark	103% of WAC, or \$40,170	Frames early Part B payment before ASP availability

Item	Planning Assumption	ARM Relevance
Estimated 340B acquisition, 23.1% assumption	\$29,991	Shows potential 340B acquisition advantage
Estimated 340B acquisition, 17.1% assumption	\$32,331	Sensitivity view if rebate assumption differs
OPPS pass-through target	Jan. 1, 2027	Planning assumption if granted
Permanent HCPCS target	Apr. 1, 2027	Planning assumption if assigned

Important limitation: These figures are planning assumptions. They should not be described as final launch facts until confirmed through final label, CMS coding decisions, pass-through status, payer policies, and updated pricing materials.

1. Default ARM Planning Frame

1.1 Primary Launch Scenario

Planning Question	Default ARM Frame
Where is launch expected to concentrate?	Pediatric specialty hospitals and centers of excellence
What site of care should be modeled first?	340B-eligible hospital outpatient department
What is the primary economic lever?	340B acquisition advantage plus payment visibility
What should non-340B represent?	Sensitivity scenario, exception case, or support-planning scenario
What should ARMs avoid?	Treating broad community-office economics as the default launch story

1.2 Why COEs Matter

Site Type	Why It Matters	ARM Translation
Pediatric specialty hospital	More likely to manage rare pediatric neurologic disease	“The right accounts already have specialized clinical infrastructure.”

Site Type	Why It Matters	ARM Translation
Academic medical center	More likely to have complex billing and payer access resources	“These sites can evaluate a sophisticated reimbursement pathway.”
Center of excellence	More likely to have NPC experience and patient concentration	“Launch adoption is likely to be concentrated, not diffuse.”
340B hospital	May have materially different acquisition economics	“340B status is central to the provider value story.”
Hospital outpatient department	Connected to OPPS and pass-through payment mechanics	“HOPD mechanics matter for payment visibility.”

2. EAP Is Not Commercial Launch Readiness

Expanded Access Program experience may show that a site can administer Adrabetadex clinically. It does not prove that the site can administer Adrabetadex commercially.

2.1 EAP Versus Commercial Launch

Dimension	EAP	Commercial Launch
Product access	Manufacturer-supported investigational access	Payer-authorized access
Product acquisition	Not a standard commercial acquisition pathway	Buy-and-bill, white-bagging, or payer-directed sourcing
Coding	Not the same as commercial billing	HCPCS, NOC, C9399, NDC, revenue code, and payer-specific logic may apply
Authorization	Not equivalent to commercial PA	Prior authorization, medical policy, and reauthorization criteria may apply
Payment	Not the central site concern	Payment amount, timing, denial risk, and reconciliation become central
Site readiness	Clinical familiarity	Billing, coding, payer, pharmacy, and revenue-cycle readiness

ARM Training Point

A site may know how to administer Adrabetadex clinically and still need support understanding commercial acquisition, coding, payment, and net-cost recovery.

3. Provider Economics Problem

The provider economics question is not whether Adrabetadex has clinical value. It is whether the site can administer a high-cost recurring therapy without creating financial loss or unacceptable working-capital exposure.

3.1 Economic Drivers

Driver	Definition	Why It Matters
Drug acquisition cost	Amount the site pays to acquire the product	Creates upfront cash exposure
Allowed reimbursement	Amount the payer allows for the drug or service	Determines gross recovery
Drug spread	Allowed drug reimbursement minus acquisition cost	Shows gross cushion before operating cost
Administration revenue	Payment for the procedure or administration service	Offsets service delivery, not necessarily drug cost
Labor cost	Physician, nursing, pharmacy, billing, and support labor	Reduces net margin
Denial / rework cost	Cost of denied, delayed, or resubmitted claims	Can erase nominal spread
Days to payment	Time from administration to payment receipt	Creates financing drag
Carrying cost	Cost of financing the acquisition exposure during delay	Important for high-cost recurring drugs
340B status	Whether the entity can acquire at 340B pricing	Can materially change economics
White-bagging	Specialty pharmacy supplies and bills for the drug	Reduces inventory risk but may remove drug spread
Medicaid PAD logic	Medicaid physician-administered drug billing and NDC requirements	Claim formatting can determine payment

3.2 Practical Site Question

Site Question	Why It Matters
Can we acquire the drug?	Determines whether buy-and-bill is operationally possible
Can we obtain authorization?	Determines whether administration can proceed
Can we code it correctly?	Determines whether the claim can be submitted cleanly
Can we document payer requirements?	Determines denial and appeal risk
Can we recover payment quickly enough?	Determines cash-flow burden
Can we preserve margin?	Determines sustainability

4. Launch Economics ARMs Should Know

4.1 Core Numbers

Figure	Calculation	Result	Meaning
WAC per administration	Fixed planning assumption	\$39,000	Gross acquisition anchor
Annual administrations	52 weeks / 2	26	Recurring treatment frequency
Annual gross WAC	$\$39,000 \times 26$	\$1,014,000	Annual patient-level drug exposure
First two quarters administrations	$26 / 2$	13	Launch-period exposure count
First two quarters gross WAC	$\$39,000 \times 13$	\$507,000	Pre-ASP gross exposure
Initial Medicare benchmark	$\$39,000 \times 103\%$	\$40,170	Early benchmark before ASP
Gross spread at 103% WAC	$\$40,170 - \$39,000$	\$1,170	Thin spread before operating costs

4.2 340B Sensitivity

Scenario	Formula	Acquisition Cost	Per-Dose Advantage Versus WAC
WAC	Fixed	\$39,000	\$0

Scenario	Formula	Acquisition Cost	Per-Dose Advantage Versus WAC
340B, 23.1% assumption	$\$39,000 \times 76.9\%$	\$29,991	\$9,009
340B, 17.1% assumption	$\$39,000 \times 82.9\%$	\$32,331	\$6,669

4.3 ARM Interpretation

Observation	ARM Translation
Gross annual exposure exceeds \$1 million per patient	Site finance and pharmacy leadership will care about cash exposure
103% of WAC creates only \$1,170 gross spread before costs	Nominal positive reimbursement can still be fragile
340B materially changes acquisition cost	340B is the main hospital economics lever
Payment delay matters	High-cost recurring therapy creates working-capital risk
Denials matter	A small denial or delay rate can materially affect net recovery

5. Coding Pathway After Approval

Adrabetadex launch payment clarity depends on two separate CMS-related concepts:

Concept	What It Does	What It Does Not Do
HCPCS coding	Identifies the product or service on claims	Does not guarantee coverage or payment
OPPS pass-through	Creates temporary Medicare HOPD payment visibility if granted	Does not override payer management or commercial policy

6. Drug Coding Definitions

This section defines the codes and claim elements ARMs may hear in provider conversations. These are **not instructions to bill**. Final code selection must be made by the provider, payer, coding team, and institutional billing policy.

6.1 Drug Codes and Claim Identifiers

Code / Element	Type	Definition	Potential Adrabetadex Relevance	ARM Caution
HCPCS	Code system	Healthcare Common Procedure Coding System, used to identify products, supplies, services, and procedures	Product-specific code would improve claim clarity once assigned	HCPCS presence does not guarantee coverage or payment
Product-specific HCPCS	HCPCS Level II	A unique code assigned to identify Adrabetadex specifically	Expected endpoint after interim coding period if assigned	Planning assumption only until CMS assigns
C9399	HCPCS Level II	Unclassified drug or biological code used in hospital outpatient billing for approved drugs without assigned codes	Potential HOPD interim pathway before product-specific HCPCS	Use only if payer/MAC/institutional billing policy supports
J3490	HCPCS Level II	Unclassified drug code	Potential professional or payer-directed interim pathway	Requires payer-specific documentation and manual pricing support
J3590	HCPCS Level II	Unclassified biologics code	Potential interim pathway if payer treats product as biologic and directs use	Do not substitute for J3490 without coding/payer support
NOC code	Coding category	Not Otherwise Classified code used when no specific code exists	May be needed before product-specific HCPCS	Often requires manual review and robust documentation
NDC	Drug identifier	National Drug Code, package-level drug identifier	Needed for Medicaid PAD, payer validation, and drug claim support	Must match actual package/vial used
Revenue code 0636	Facility revenue code	Hospital outpatient revenue code for drugs requiring detailed coding	May apply to HOPD detailed drug billing	Confirm institutional and payer rules

Code / Element	Type	Definition	Potential Adrabetadex Relevance	ARM Caution
Type of Bill 13x	Institutional billing field	Hospital outpatient claim bill type series	Relevant for UB-04 / 837I HOPD claims	Facility-specific billing workflow applies
UB-04 / 837I	Institutional claim format	Claim format used by hospitals and facilities	Relevant to HOPD billing	Facility revenue-cycle team controls
CMS-1500 / 837P	Professional claim format	Claim format used by physicians and professional suppliers	Relevant to physician or professional billing	Professional billing rules differ from facility billing
PAD	Medicaid drug billing category	Physician-administered drug	Medicaid may require HCPCS plus NDC reporting	State and MCO requirements vary

7. Interim Coding Before Product-Specific HCPCS

7.1 Interim Coding by Setting

Setting	Potential Interim Pathway	Documentation Emphasis	ARM Message
HOPD	C9399 or payer-directed NOC logic	NDC, dose, route, units, invoice, medical necessity	"Interim HOPD claims need clean documentation."
Physician office	J3490, J3590, or payer-directed miscellaneous code	Drug name, NDC, dose, route, units, invoice, PA	"Professional billing may need manual-pricing support."
Commercial payer	Payer-specific medical policy and sourcing instructions	PA, site of care, sourcing rule, NDC, invoice	"Do not assume Medicare logic controls commercial payment."
Medicaid	PAD claim logic with NDC reporting	HCPCS/NOC, NDC, NDC unit, PA, 340B status	"Claim format can determine payment."

7.2 HOPD Interim Billing Elements

Billing Element	Definition	Typical Need	Risk if Wrong
Type of Bill	Identifies institutional claim type	13x hospital outpatient	Claim routing or adjudication issue
Revenue Code	Facility revenue category	0636 for detailed drug coding where applicable	Claim edit or mismatch
HCPCS	Drug/product/service code	C9399 or payer-directed interim code	Manual review or denial
NDC	Package-level drug identifier	Actual vial/package NDC	Medicaid or payer validation failure
Units	Quantity billed	May differ by code and payer	Incorrect payment or denial
Drug name	Product name	Supports manual review	Manual pricing delay
Dose administered	Actual dose given	Supports medical record and claim	Audit or payment issue
Route	How drug was administered	Intrathecal administration context	Medical necessity or coding issue
Invoice	Acquisition-cost support	Often needed for NOC/manual pricing	Payment delay or underpayment
PA approval	Prior authorization approval	Needed when payer requires	Denial risk
Medical necessity	Clinical rationale	Supports PA, claim, appeal, continuation	Denial or appeal weakness

8. Billing and Coding Reference Path

This section directs detailed code, billing, and modifier content to the Billing Reference reference document for detailed definitions and table-level guidance.

For ARM narrative use only payer-directed, candidate-safe language and confirm any billing or coding element with the provider's billing and pharmacy team before use.

12. OPPS Pass-Through

12.1 OPPS and Pass-Through Definitions

Term	Definition	ARM Translation
OPPS	Medicare Outpatient Prospective Payment System for hospital outpatient services	HOPD payment framework
Transitional pass-through	Temporary additional Medicare payment for qualifying new drugs, biologicals, and devices	Payment visibility bridge
Separately payable	Product or service may be paid separately rather than packaged	Helps hospitals evaluate drug recovery
Pass-through period	Temporary window, generally at least 2 years and not more than 3 years for qualifying drugs/biologicals	Temporary support, not permanent reimbursement

12.2 What Pass-Through Does and Does Not Do

Pass-Through Does	Pass-Through Does Not Do
Improves Medicare HOPD payment visibility	Does not guarantee payment
Helps bridge launch-period outpatient payment uncertainty	Override commercial payer rules
Supports hospital outpatient planning	Remove prior authorization
Creates a defined Medicare outpatient payment pathway if granted	Eliminate white-bagging or specialty pharmacy sourcing
May improve site confidence	Solve Medicaid billing mechanics
Supports reimbursement-readiness narrative	Remove documentation requirements

12.3 Pass-Through Timing

Milestone	Planning Date	Status
PDUFA / approval assumption	Aug. 17, 2026	Planning date
Pass-through package target	Sept. 2026	Planning assumption
Earliest practical pass-through effective date	Jan. 1, 2027	If granted
Permanent HCPCS target	Apr. 1, 2027	If assigned

13. 340B Economics

13.1 340B Definition

340B is a federal drug pricing program that allows eligible covered entities to purchase covered outpatient drugs at or below a statutory ceiling price, subject to program requirements.

13.2 Why 340B Matters

Scenario	Acquisition	Payment Benchmark	Gross Drug Spread	Annualized Spread
Commercial buy-and-bill at WAC	\$39,000	\$39,000	\$0	\$0
Medicare initial-sales benchmark	\$39,000	\$40,170	\$1,170	\$30,420
340B, 23.1% assumption	\$29,991	\$40,170	\$10,179	\$264,654
340B, 17.1% assumption	\$32,331	\$40,170	\$7,839	\$203,814
White-bagging	\$0 provider acquisition	\$0 provider drug reimbursement	\$0	\$0

13.3 ARM Language

Use This	Avoid This
"340B can materially change acquisition economics."	"340B guarantees profit."
"Compliance and payer mix still matter."	"340B solves reimbursement."
"Each site must apply its own 340B billing controls."	"All 340B sites will have the same result."
"Duplicate-discount controls are important in Medicaid."	"Medicaid 340B is simple."

14. White-Bagging and Specialty Pharmacy

14.1 Sourcing Model Definitions

Model	Definition	Provider Economic Impact
Buy-and-bill	Provider acquires drug, administers it, and bills payer for drug and service	Inventory risk remains, but provider may retain drug spread
White-bagging	Specialty pharmacy ships drug to provider for patient-specific administration and bills payer for drug	Inventory risk may fall, but provider drug reimbursement/spread may be removed
Specialty pharmacy sourcing	Payer directs drug sourcing through specialty pharmacy	Provider may be limited to administration billing
Mixed model	Different payers require different sourcing pathways	Requires payer-by-payer scenario modeling

14.2 White-Bagging Discussion Guide

Provider Question	ARM Response
Can we avoid inventory risk?	Possibly, depending on payer sourcing rules.
Do we still make margin?	Drug spread may be reduced or removed.
Is white-bagging always better?	Not necessarily, operational complexity may increase.
Can we choose buy-and-bill?	Only if payer policy and contracting permit it.
What if sourcing rules are not followed?	Claim denial or nonpayment risk may increase.

15. Medicaid and Pediatric Access

15.1 Medicaid Definitions

Term	Definition	Why It Matters
Medicaid FFS	State-administered fee-for-service Medicaid	State payment rules control
Medicaid managed care	Medicaid administered through contracted MCOs	MCO policies may differ from state FFS

Term	Definition	Why It Matters
EPSDT	Early and Periodic Screening, Diagnostic, and Treatment benefit for Medicaid beneficiaries under 21	Supports pediatric medical necessity
PAD	Physician-administered drug	Often requires HCPCS plus NDC reporting
NDC	National Drug Code	Required for many Medicaid PAD claims
Carve-in	340B drug used for Medicaid claim with appropriate billing controls	Duplicate-discount controls required
Carve-out	340B drug not used for Medicaid claim	Different acquisition and billing logic
Duplicate discount	Same unit receives both 340B discount and Medicaid rebate	Prohibited compliance risk

15.2 Medicaid ARM Frame

Medicaid Issue	ARM Message
EPSDT	Supports pediatric medical necessity, but does not create one national payment formula
State variation	Payment and billing rules vary by state
Managed care	MCO requirements may differ from FFS
NDC reporting	Incorrect NDC or units can block payment
340B status	Carve-in/carve-out logic must be clear
Duplicate discounts	Compliance controls are required
Denials and timing	Payment delay and rework can affect net recovery

16. Calculator Use

16.1 Calculator Purpose

The calculator is a conversation tool. It does not guarantee coverage, coding, payment, or margin.

16.2 Calculator Inputs

Input	Definition	Why It Matters
Site type	HOPD, COE, office, hospital, or other setting	Determines coding and payment context
340B status	Whether site uses 340B acquisition	Determines acquisition economics
Payer type	Commercial, Medicare, Medicaid FFS, Medicaid MCO	Determines reimbursement and control assumptions
Sourcing model	Buy-and-bill, white-bagging, specialty pharmacy	Determines whether provider bills for drug
Coding state	Interim NOC, pass-through, permanent HCPCS, ASP-based	Determines claim clarity and payment benchmark
WAC	List-price benchmark	Establishes baseline exposure
Allowed amount	Payer payment benchmark	Determines gross recovery
Denial rate	Expected claim denial rate	Models rework and payment risk
Days to payment	Expected payment lag	Models financing drag
Labor/admin cost	Non-drug operating cost	Determines net margin
Manufacturer support	Support amount, if any	Can stabilize fragile scenarios

16.3 Calculator Outputs

Output	Definition	Use
Drug spread	Allowed drug reimbursement minus acquisition cost	Shows gross drug recovery
Operating margin	Net after operating costs and risk adjustments	Shows sustainability
Annualized margin	Per-dose margin across 26 administrations	Shows recurring patient-level result
Cash exposure	Amount carried before payment	Shows working-capital burden

Output	Definition	Use
Break-even support	Amount needed to neutralize negative economics	Supports support-program planning
Risk flags	Qualitative markers for margin, cash, and operational risk	Helps ARMs interpret results

17. ARM Conversation Flow

Step	ARM Action	Key Question
1	Confirm site type	Is this a COE, HOPD, pediatric hospital, office, or other site?
2	Confirm 340B status	Is the site 340B eligible and using 340B for this pathway?
3	Confirm payer mix	Commercial, Medicaid FFS, Medicaid MCO, Medicare, other?
4	Confirm sourcing	Buy-and-bill, white-bagging, specialty pharmacy, mixed?
5	Confirm coding state	Interim code, pass-through active, permanent HCPCS, ASP-based?
6	Run calculator	What is per-dose and annualized net recovery?
7	Interpret result	Is the pathway sustainable, fragile, or support-dependent?

18. Scenario Interpretation Table

Scenario	Likely Interpretation
340B HOPD, pass-through active	Strongest pathway if claims and compliance are clean
340B HOPD, pre-pass-through	Acquisition economics may be favorable, but coding/payment uncertainty remains
340B HOPD, permanent HCPCS active	Cleaner claims, but payer controls still matter
Non-340B commercial buy-and-bill	Potentially fragile unless payment is strong and denials are low
Medicaid non-340B	High risk of negative economics depending on state payment
Medicaid 340B COE	Potentially stronger economics, but duplicate-discount controls are central

Scenario	Likely Interpretation
White-bagging	Lower inventory exposure, but drug spread may be removed
Specialty pharmacy mandated	Provider may retain administration work without drug reimbursement

19. Do / Do Not Say

Do Say	Do Not Say
"The model quantifies provider economics under different scenarios."	"The model guarantees reimbursement."
"340B can materially change acquisition cost."	"340B guarantees profit."
"Pass-through can improve Medicare HOPD predictability."	"Pass-through guarantees payment."
"Coding readiness matters during launch."	"Permanent HCPCS solves reimbursement."
"White-bagging can reduce inventory risk but remove drug spread."	"White-bagging solves the economic issue."
"Medicaid must be evaluated separately."	"Medicaid will follow commercial logic."
"EAP experience does not equal commercial readiness."	"EAP means commercial use will be easy."
"Code selection must be verified by provider coding and payer policy."	"This is the correct code in all cases."

20. One-Page ARM Summary

Topic	ARM Summary
Core point	Adrabetadex adoption depends on provider economics and operational readiness, not coverage alone
Default account type	COE / pediatric specialty hospital
Default setting	340B-eligible HOPD
Key exposure	\$39,000 per dose, \$1.014M annual gross WAC

Topic	ARM Summary
Main economic lever	340B acquisition advantage
Main launch bridge	Interim coding plus pass-through planning
Main mature-state improvement	Product-specific HCPCS, if assigned
Main commercial risk	PA, site-of-care controls, white-bagging, sourcing rules
Main Medicaid risk	State payment, NDC reporting, managed care variation, duplicate-discount controls
Calculator role	Scenario-specific provider economics discussion tool

21. Key Takeaways

Takeaway	Meaning
Adoption is COE-led	The first-wave story should focus on specialty centers, not broad community use
340B is central	It materially changes acquisition economics
Coverage is not enough	Sites still need payment, coding, sourcing, and cash-flow clarity
EAP is not commercial readiness	Commercial launch adds payer and claim mechanics
HCPCS and pass-through are separate	One is claim identification, the other is temporary HOPD payment support
Interim coding matters	NOC/unclassified claims require documentation discipline
White-bagging changes economics	It may lower inventory risk but remove drug spread
Medicaid is distinct	EPSDT supports coverage arguments, but payment mechanics remain state-specific
The calculator should lead with scenarios	Site type, payer, 340B, sourcing, and coding state drive the discussion